HEALTHCARE OF COMMUNITY

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WHY THIS CHAPTER?

FOR EXAMINATION

1. High yielding chapter.
2. Question repeats often from this chapter.

MUST KNOW AS A DOCTOR

1. Gives overall knowledge about health policies of India.
2. Provides idea about various levels of infrastructure of health care.

TOPICS OF DISCUSSION UNDER THIS CHAPTER

1. PRIMARY HEALTH CARE
2. JOB DESCRIPTION OF MULTIPURPOSE HEALTH WORKER (FEMALE)
3. JOB RESPONSIBILITIES OF ASHA
4. COMMUNITY HEALTH CENTER

PRIMARY HEALTH CARE

INTRODUCTION

- This concept was proposed by WHO.
- Came into existence in 1978 following an international conference at Alma Ata.

DEFINITION

Primary health care is **essential healthcare** made **universally accessible** to individuals & **acceptable** to them through their **full participation** & at a cost the **community & country can afford.**

ELEMENTS/COMPONENTS

(Pneumonic – ELEMENTS)

E - L E - M E N - S

E Education about prevailing health problems, & methods of preventing & controlling them.

L - Locally endemic diseases – prevention & control.

E - Essential drugs.

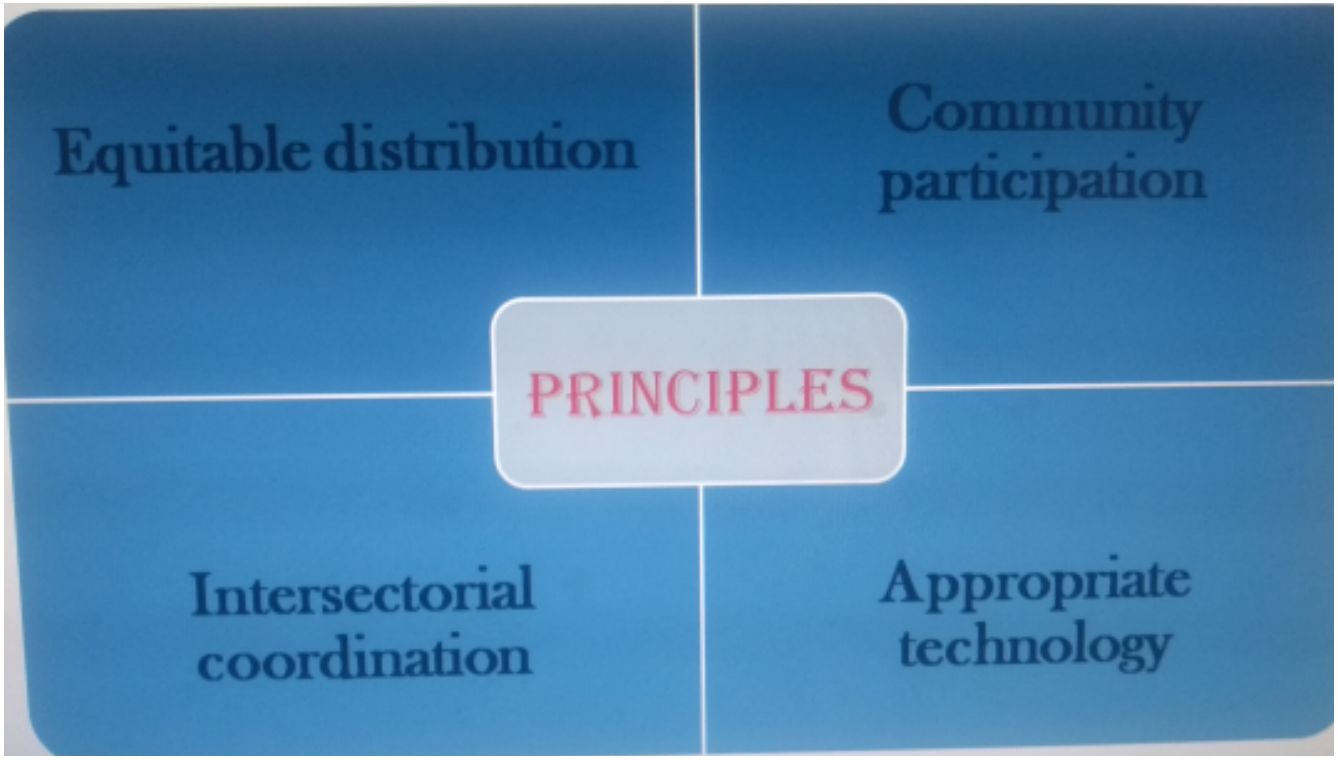
M MCH including family planning.

E - EPI against vaccine preventable diseases.

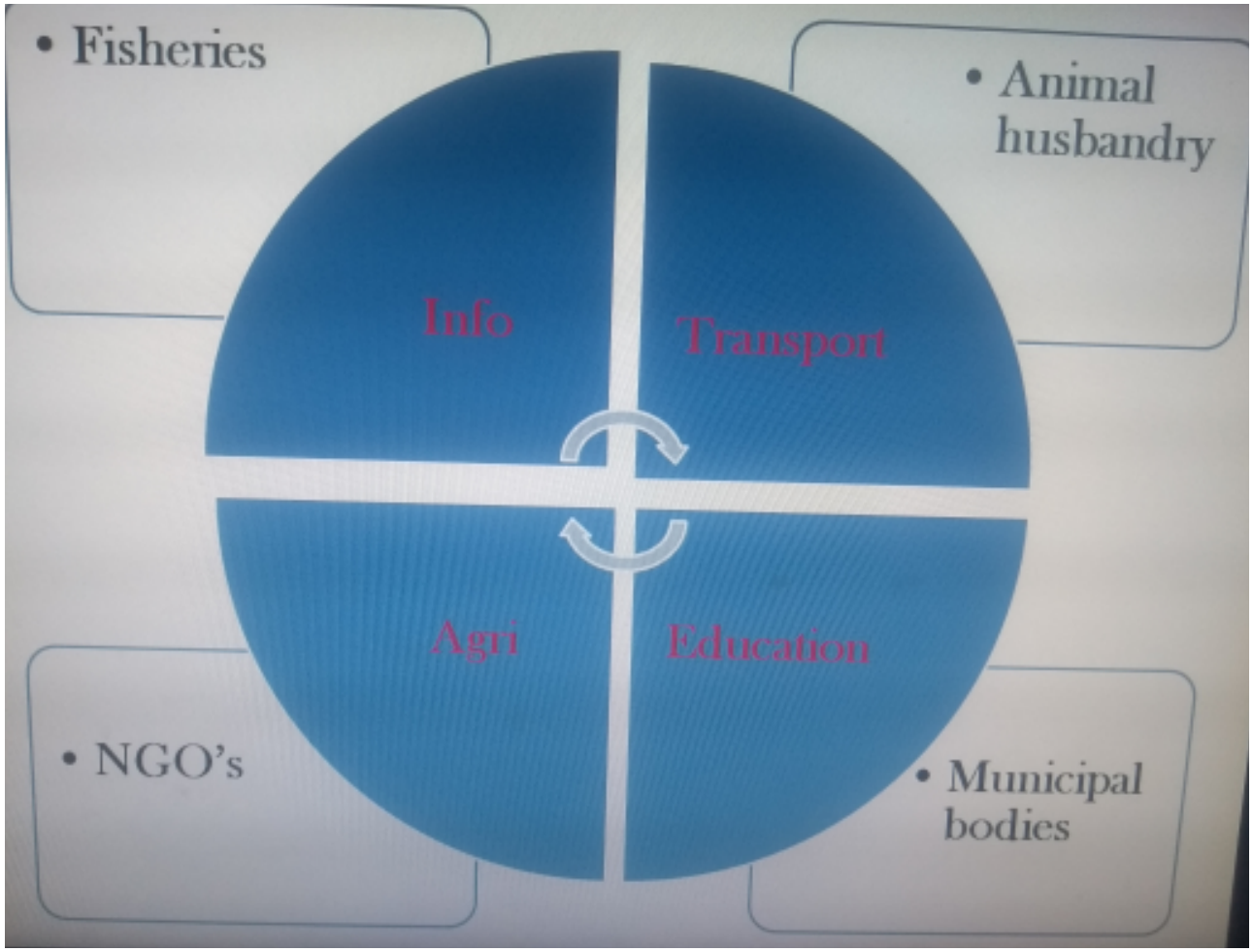
N Nutrition & promotion of food supply.

T - Treatment of common diseases & injuries

S Safe water supply & basic sanitation


Equitable distribution
Community participation
PRINCIPLES
Intersectorial coordination
Appropriate technology


Equitable distribution
• Irrespective – rich or poor.
• Fight negligence on poor rural & urban slum people.
Community Participation
• Promotion of their own health & welfare.
• Self reliance of community.
• Community acceptance of primary health care programme.
• Eg – trained dais.


• Fisheries
• Animal husbandry
Info
Transport
Agri
Education
• NGO's
• Municipal bodies

APPROPRIATE TECHNOLOGY

Examples	Elements
• Scientifically sound. • Effective and simple to use. • Acceptable to those who use. • Easily affordable to community.	• Oral rehydration solution. • Immunization program. • Distribution of iron and folic acid. • DOTS.

SIGNIFICANCE OF PRIMARY HEALTHCARE

- ‘ Health by people, placing people’s health in people’s hands’.
- Hallmark’s of Primary health Care

1. Affordibility
2. Acceptability
3. Accessability
4. Availability

MULTIPURPOSE HEALTH WORKER (FEMALE)- JOB DESCRIPTION

INTRODUCTION

- Auxiliary nurse midwife
- Posted in sub-center

JOB DESCRIPTION

1. Record keeping:
   - Antenatal register
   - Postnatal register
   - Under five children register
   - Eligible couple register
   - Family planning registers
   - Birth & death registers

2. Antenatal care:

- Care at home – 4-5 visits to expectant mother
- Care at clinic- iron & folic acid tabs

TT injections.

3. Intranatal care:

- 50% of total deliveries
- Supervises deliveries- dias
- Danger signals during labor- refers

4. Postnatal care:

- 3-5 visits – newly delivered mother – home
- Nutrition , breastfeeding , personal hygiene, family planning.

5. Family planning services:

- Newly married couples- oral pills, contraceptives
- Eligible couples- IUD insertion
- Target couples – sterlization
- Identifies cases for MTP

6. **Care of children** :

- **growth & development, health card.**
- **Arranges immunization**
- **Vitamin A solution**

7. **Notification**

- **Notifiable diseases**
- **outbreaks**

8. Training
- Dias
- Village health guides

9. Health eduction

10. Healthcare
- Urine , Hb%
- ORS packets, first aid, minor ailment.

Significance:
- Created on recommendation of Shrivastav committee
- ‘Face of health care delivery’ in rural areas.

JOB RESPONSIBILITIES OF ASHA

INTRODUCTION

- ASHA – Accredited social health activist
- Cadre under NRHM
- Selected by village health & sanitation committee of Panchayat
- 1 ASHA – 1000 population- plain areas

  1 ASHA – 1 Habitat – hilly areas
- Works under guidence of – auxillary midwife, anganawadi

SELECTION CRITERIA

- Women either married, widow or divorced.
- Resident of village
- Age preferably 25-45 years
- Formal education upto 8th class
- Communication skills & leadership qualities
- Disadvantaged population group

ROLE

- Info – nutrition, basic sanitation & hygiene, utilization of health & family welfare services
- Promote good health services, timely referrals
- Counsel pregnant women
- Part of Janani suraksha yojana – to prevent MMR
- Escort pregnant women & children
- DOTS Agent

- Depot holder
- Notifying outbreaks, births & deaths
- Construction of household toilets

MONITORING & EVALUATION OF ASHA

Process indicators	Outcome indicators	Impact indicators
No. of ASHA'S selected No. of ASHA'S trained Review meeting	% of JSV claim's made % of newborn's weighed & families counselled % of deliveries with skilled assistance	IMR Child malnutrition rates No. of cases of TB/ leprosy detected compared to last year

SIGNIFICANCE

- Imp. Link between community & public health care services

COMMUNITY HEALTH CENTER

Community health Centres

- One CHC for 1.20 lakh population in plain area and 80,000 population in hilly, tribal difficult area
- As per minimum norms, a CHC is required to be manned by four Medical Specialists i.e. Surgeon, Physician, Gynecologist and Pediatrician supported by 21 paramedical and other staff.
- It has 30 in-door beds with one OT, X-ray, Labor Room and Laboratory facilities.
- It serves as a referral centre for 4 PHCs and also provides facilities for obstetric care and specialist consultations.
- As on March, 2014, there are 5,363 CHCs functioning in the country.

Staff for community Health Centre :

- Medical officer 4
- Nurse mid-wives 7
- Dresser 1
- Pharmacist/Compounder 1
- Lab. technician 1
- Radiographer 1
- Ward boys 2
- Sweepers 3
- Other health workers: Dhobi - 1, Mali - 1, Chowkidar -1, Aya -1, Peon - 1.

In total 25

Functions of Community Health Centre

- To provide specialist services (routine and emergency cases in surgery, medicine, obs & gyn. and Paediatrics) .
- Indoor patient care (30 beds are available).
- RCH services.
- Services for National Health Programmes.
- Cold chain maintenance through ILR and deep-freezers, cold boxes, and day carriers.
- Laboratory and x-ray services.
- Blood storage facility.
- Referral Services
- Training and continuing education of health teams.
- Information, education and communication (IEC)activities for specific problems.
- To elicit community participation.
- To enhance public private partnership(PPP).

QUIZ TIMÆ !

1. PRIMARY HEALTH CARE WAS FIRST PROPOSED BY

1. SAARC
2. WHO
3. ICMR
4. USA

ANSWER

WHO

2.FEMALE MULTIPURPOSE HEALTH WORKER IS A

1. Dai
2. Auxiliary nurse midwife
3. Anganawadi worker
4. Village health guide

ANSWER

Auxillary nurse midwife

3. ASHA IS SELECTED BY

1. Dept. of health & family welfare
2. District administration
3. Village health & sanitation committee of panchayat
4. MO of PHC

ANSWER

3. **Village health & sanitation committee of panchayat**

4. WHAT IS THE SCHEDULED DOSE OF BELOW GIVEN CONTRACEPTIVE?



ANSWER

1.5 mg within 5 days of unprotected sex

5.WHICH IS NOT TRUE ABOUT CHC?

1. Upgraded PHC
2. Covers population of 2 lakh
3. First referral unit
4. Specialists services are available

ANSWER

2. covers population of 2 lakhs

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